Panitumumab

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · antibody monotherapy

Active medicines: Panitumumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg PO once	Before panitumumab, according to the individual antiemetic prescription NCIS V8 low-risk row. Routine infusion-reaction premedication is a separate issue and is not usually required for panitumumab.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- No routine cetuximab-style antihistamine/steroid premedication is imported into panitumumab.
- Provide local EGFR skin care, periodic magnesium/calcium/potassium checks and monthly post-treatment electrolytes for two months.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00225 v6 · p3 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-05 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required